

Veeva Vault MedInquiry

Medical Inquiry — Complete Reference Guide

[Concepts](#) · [Object Structure](#) · [Configuration](#) · [Implementation](#) · [Best Practices](#)

CONCEPTS

OBJECT MODEL

CONFIGURATION

IMPLEMENTATION

PURPOSE	A complete, detailed reference for Vault MedInquiry covering every concept, object, field, configuration step, and implementation consideration. Validated against Veeva product documentation and real-world implementations.
AUDIENCE	Vault Admins, Medical Information Specialists, Pharmacovigilance teams, Medical Affairs leadership, Business Analysts, Implementation Consultants.
COVERS	What is Medical Inquiry · Business Process · Object Model · Field Reference · Workflows & Lifecycles · FAQ Auto-Detection · Adverse Event Handling · Configuration Steps · Implementation Planning · System Architecture

1. What is a Medical Inquiry?

Definition

A **Medical Inquiry (MI)** — also called a **Medical Information Request (MIR)** — is any unsolicited or solicited request for information about a pharmaceutical product received from a Healthcare Professional (HCP), patient, caregiver, payer, or internal staff member such as a sales representative or MSL.

Who Sends Medical Inquiries?

- **Healthcare Professionals (HCPs):** Doctors, pharmacists, nurses, dentists, specialists
- **Patients or Caregivers:** Patients asking about their own medication
- **Payers / Formulary Committees:** Insurance bodies evaluating a drug for formulary
- **Internal Staff:** Sales reps or MSLs relaying questions from HCP meetings via CRM

Types of Questions That Come In

Category	Example Question	Why It Matters
Dosing	What dose of Drug X is safe for a 75-year-old with eGFR of 25?	Incorrect advice causes patient harm
Drug Interactions	Can Drug X be combined with warfarin and metformin simultaneously?	Interactions can be life-threatening
Pregnancy / Lactation	Is Drug X safe during the second trimester?	Teratogenicity risk — high clinical sensitivity
Off-label Use	Has Drug X been studied in paediatric patients under 12?	Must respond carefully — cannot promote off-label
Adverse Events	My patient developed jaundice 2 weeks after starting Drug X	Legal obligation to capture and report to PV
Product Quality	The tablets I received look a different colour	Capture as PQC and transfer to Quality team
Mechanism of Action	How exactly does Drug X lower blood pressure?	Scientific education — answer from approved content
Storage / Stability	Drug X was left out of the fridge for 48 hours — still safe?	Patient safety — requires precise approved answer

■ **Regulatory obligation:** Companies must capture ALL medical inquiries — even routine ones. Any mention of a patient experiencing a harmful effect is an Adverse Event that **MUST** be reported to regulatory authorities. Serious AEs: 15 calendar days. Non-serious AEs: periodic reports. Missing a report can result in warning letters or loss of marketing authorization.

2. The Business Process

Why a Medical Inquiry System is Required

Medical Affairs teams at pharmaceutical companies are legally and ethically required to: receive and respond to all medical questions; provide accurate, approved, balanced information; capture every interaction in a permanent auditable record; report all Adverse Events to Pharmacovigilance; and meet defined SLA response times.

Who Works in the Medical Information Team?

Role	Responsibility in MedInquiry
MI Specialist	Front-line: answers phone/email, creates Cases, finds answers, sends responses
Senior MI Specialist	Handles complex / escalated inquiries, approves non-standard responses
MI Manager / Supervisor	Oversees SLA compliance, quality, approves responses before sending
Medical Writer	Creates and maintains Standard Response Documents (SRDs) and FAQ library
Pharmacovigilance Specialist	Receives AEs transferred from MedInquiry, processes in Safety Vault
Medical Science Liaison	Field-based: submits inquiry requests from HCP meetings via CRM

Real-Life Scenario — Why This Matters

Dr. Patel calls the medical information line asking about Drug X dosing. During the call she mentions a patient was hospitalized after taking Drug X. The specialist answers the dosing question but forgets to formally capture the hospitalization as an Adverse Event. Three weeks later the FDA requests all serious AE reports. That report is missing — a regulatory failure.

With MedInquiry: The specialist opens the Case Intake form when the call connects, answers the dosing question using a pre-approved Standard Response, captures the hospitalization as an AE in the same form, and transfers it to the Safety team in one click. The FDA audit passes because every AE is in the system with a full audit trail.

Problems Before MedInquiry

Problem	Real-World Consequence
Inquiries in shared email inbox – no tracking	Missed inquiries, inconsistent answers, no audit trail
AEs transferred to safety manually via email	Data entry errors, missed AEs, regulatory reporting delays
SRDs in shared drives with no version control	Outdated or unapproved responses sent to HCPs

Problem	Real-World Consequence
SLA compliance tracked in spreadsheets	SLAs missed, no defensible compliance audit trail
No keyword monitoring	Spike in safety queries unnoticed — delayed signal detection
Weekly MI/PV reconciliation meetings	2+ hours wasted weekly (Teva eliminated this entirely)

3. Inquiry Channels

How Inquiries Arrive — All Channels

MedInquiry captures inquiries from every channel into the same Case structure. All channels create the same type of Case — no channel produces a different record type.

Channel	How It Works in MedInquiry	Key Integration
Phone	Genesys Cloud telephony: call accepted → Case Intake form opens automatically in browser. Caller phone number may pre-populate Contact.	Genesys Cloud Embeddable Framework
Email	Monitored inbox connected to MedInquiry. Incoming emails auto-create Case records with email body in Case Request question field.	Email ingestion configuration
Website	Web form on company medical portal submits via REST API, auto-creating Case + Case Request records.	Vault REST API (POST /objects/case__v)
Veeva CRM	MSLs and reps submit inquiry requests from CRM. Auto-creates Cases in MedInquiry — no duplicate entry required.	Medical-CRM Vault Connection
Manual	Specialist manually creates a Case for walk-in visitors, postal letters, or congress interactions.	No integration — manual UI entry

4. Complete Object Model

Object Hierarchy

MedInquiry uses Vault's relational object model. Every piece of inquiry data is stored in a specific object. All objects are linked hierarchically with parent-child relationships.

case__v	ROOT PARENT — One per inquiry interaction
person__v	Case Contact — HCP or patient who asked
case_request__v	One per question asked within the Case
faq__v	Auto-detected matching FAQ
standard_response__v	Pre-approved answer linked to FAQ
case_response__v	The response being assembled
fulfillment_doc__v	Each document to include in response package
event__v (adverse_event__v)	Adverse Event captured during inquiry
event_product__v	Drug(s) involved in the AE
event_reaction__v	Reaction(s) experienced by patient
case_sla__v	SLA deadline calculation rules

case__v — The Case Object (Root Parent)

Field (API Name)	Type	Purpose
name__v	Text	Auto-generated case number e.g. MI-2026-04521
status__v	Picklist	Lifecycle state: new__v / in_progress__v / pending__v / fulfilled__v / closed__v
priority__v	Picklist	urgent__v / high__v / normal__v / low__v — drives SLA deadline calculation
channel__v	Picklist	How inquiry arrived: phone__v / email__v / web__v / crm__v / manual__v
case_contact__v	Lookup → person__v	Links Case to the HCP or patient who asked
product__v	Lookup → Product	Which drug the inquiry is about
country__v	Lookup → Country	Country — drives FAQ matching and SLA rules
language__v	Picklist	Language of inquiry — drives FAQ matching
sla_deadline__v	DateTime	Auto-calculated deadline — when Case must be fulfilled
assigned_to__v	Lookup → User	Which specialist is responsible for this Case
notepad__v	Long Text	Internal specialist notes — NOT included in response to HCP

Field (API Name)	Type	Purpose
awareness_date__v	Date	When company became aware — starts regulatory AE reporting clock

case_request__v — The Question

Field (API Name)	Type	Purpose
case__v	Parent Lookup	Links question to parent Case
question__v	Long Text	The actual question text — used by FAQ auto-detection matching algorithm
product__v	Lookup → Product	Product the question is about — used as filter in FAQ matching
category__v	Picklist	Dosing / Drug Interaction / Safety / Efficacy / Off-Label / MOA / Storage
language__v	Picklist	Language of question — used as filter in FAQ matching
country__v	Lookup → Country	Country — used as filter in FAQ matching
suggested_faq__v	Lookup → faq__v	The FAQ auto-detection matched — populated automatically by system
keyword_tags__v	Multi-Picklist	Keywords automatically applied by Keyword Tracking scan

case_response__v — The Answer

Field (API Name)	Type	Purpose
case__v	Parent Lookup	Links response to parent Case
response_notes__v	Rich Text	Approved answer text — auto-populated from Standard Response on FAQ match
cover_letter_template__v	Lookup	Which template to use for personalized cover letter
send_method__v	Picklist	secure_link__v / email_attachment__v / postal__v / fax__v
send_status__v	Picklist	draft__v / pending_approval__v / sent__v / delivered__v
sent_date__v	DateTime	When response was sent — evidence for SLA compliance

fulfillment_doc__v — Documents to Include

Field (API Name)	Type	Purpose
case_response__v	Parent Lookup	Links document to its parent Case Response
document__v	Lookup → Document	The actual MedComms SRD or medical letter document to include
order__v	Number	Sequence in response package — Document 1, 2, 3...
required__v	Checkbox	If true, document is mandatory in response — cannot be removed

event__v — Adverse Event

Field (API Name)	Type	Purpose
case__v	Parent Lookup	Links AE to parent Case
object_type__v	Object Type	adverse_event__v OR product_quality_complaint__v
serious__v	Checkbox	Is this a Serious AE? Serious = 15-day expedited reporting required
awareness_date__v	Date	Date company first became aware — starts regulatory reporting clock
patient_age__v	Number	Patient age at time of event
patient_sex__v	Picklist	male__v / female__v / unknown__v
patient_weight__v	Number	Patient weight in kg
transfer_status__v	Picklist	not_transferred__v / transferred__v — updated after Medical-Safety Connection fires
safety_case_number__v	Text	Safety case number assigned by Safety Vault — populated via reconciliation

event_product__v — Drug Involved

Field (API Name)	Type	Purpose
event__v	Parent Lookup	Links to parent AE event
product_name__v	Text	Name of the drug involved
dose__v	Number	Dose the patient was taking
dose_unit__v	Picklist	mg / mcg / ml / IU / etc.
frequency__v	Picklist	once_daily__v / twice_daily__v / weekly__v / as_needed__v
route__v	Picklist	oral__v / intravenous__v / subcutaneous__v / topical__v
start_date__v	Date	When patient started the drug — establishes temporal relationship to reaction
end_date__v	Date	When patient stopped the drug
action_taken__v	Picklist	drug_withdrawn__v / dose_reduced__v / dose_not_changed__v / unknown__v

event_reaction__v — The Reaction

Field (API Name)	Type	Purpose
event__v	Parent Lookup	Links to parent AE event
reaction_description__v	Text	Free text description in reporter's own words
meddra_pt__v	Text	MedDRA Preferred Term — standardized medical terminology code for the reaction
severity__v	Picklist	mild__v / moderate__v / severe__v / life_threatening__v / fatal__v
outcome__v	Picklist	recovering__v / recovered__v / not_recovered__v / recovered_with_sequelae__v / fatal__v / unknown__v
start_date__v	Date	When the reaction started
end_date__v	Date	When the reaction resolved (if it did)

faq__v — Frequently Asked Questions

Field (API Name)	Type	Purpose
name__v	Text	Internal name e.g. "Drug X — Renal Dosing FAQ"
question__v	Long Text	Question text used by auto-detection algorithm — write as HCP would phrase it
product__v	Lookup → Product	Scope — only Case Requests for this product trigger this FAQ
country__v	Lookup → Country	Country scope — blank = global, populated = country-specific
language__v	Picklist	Language — must match Case Request language to be suggested
category__v	Picklist	Dosing / Safety / Efficacy / MOA / Off-Label / Storage
status__v	Lifecycle	Must be Active for auto-detection to use it

standard_response__v — Pre-Approved Answer

Field (API Name)	Type	Purpose
faq__v	Parent Lookup	Links answer to parent FAQ question
response_notes__v	Rich Text	Approved answer text — auto-copied to Case Response when specialist accepts
language__v	Picklist	Response language — one FAQ can have responses in many languages
country__v	Lookup	Country-specific response — one FAQ can have different answers per country
status__v	Lifecycle	Must be Active to be suggested by auto-detection

case_sla__v — SLA Response Time Rules

Field (API Name)	Type	Purpose
priority__v	Picklist	Which Case priority this SLA rule applies to
response_time__v	Number	How many units the team has to respond
time_unit__v	Picklist	hours__v OR business_days__v
business_hours__v	Checkbox	If true: counts only business hours. False: counts calendar hours/days.
country__v	Lookup	Country-specific SLA — different markets may have different regulatory requirements

5. Lifecycles and Workflows

Case Lifecycle States

State	Meaning	Who Triggers Transition
New	Case just created — unassigned	System (auto-creation) or Supervisor assigns
In Progress	Specialist is actively working on response	Assigned Specialist
Pending	Waiting — HCP callback, more info, internal approval	Assigned Specialist
Fulfilled	Response has been sent to the HCP	Specialist after sending Case Response
Closed	Case fully complete including AE transfer	Specialist or Supervisor

Case Response Lifecycle

State	Meaning
Draft	Being assembled — not yet reviewed or sent
Pending Approval	Optional — awaiting Supervisor review before sending
Approved to Send	Supervisor approved — ready to send to HCP
Sent	Response delivered to HCP — triggers Case transition to Fulfilled

Key Entry Actions on MedInquiry Lifecycles

Object	State	Entry Action	What It Does
Case	New	Send Notification	Notifies supervisor of new unassigned Case
Case	In Progress	Calculate SLA Deadline	Sets sla_deadline__v from Case SLA rules
Case	Fulfilled	Send Notification	Sends confirmation to Case Contact that response sent
Case Response	Pending Approval	Start Workflow	Supervisor Approval workflow — review task assigned
Case Response	Sent	Update Case Status	Triggers Case to transition to Fulfilled automatically
Adverse Event	Transferred	Send Notification	Notifies PV team that new AE has been transferred

6. FAQ Auto-Detection

How FAQ Auto-Detection Works

FAQ auto-detection is the most important efficiency feature in MedInquiry. When a specialist saves a Case Request, the system immediately tries to find a pre-approved answer from the FAQ library.

- 1 Case Request Saved**
Specialist creates Case Request with question text, product, country, language.
- 2 Filter FAQs by Scope**
MedInquiry filters FAQ library to only: product matches AND country matches AND language matches AND status = Active.
- 3 Text Similarity Comparison**
Case Request question__v text is compared against filtered FAQ question__v fields using a similarity algorithm. Score 0-100% calculated for each.
- 4 Threshold Check**
If highest score exceeds threshold (typically 70-75%), that FAQ is selected. Multiple matches above threshold: highest score wins.
- 5 Suggestion Displayed**
Suggested FAQ + Standard Response shown to specialist with similarity score. Specialist: Accept / Reject / Ignore.
- 6 Case Response Auto-Populated**
On Accept: Standard Response response_notes__v copied to Case Response. All standard_response_doc__v child records copied as fulfillment_doc__v records. Package ready to send.

Auto-Detection Troubleshooting Guide

Symptom	Most Likely Cause	Fix
No suggestions ever appear	FAQ Auto-Detection not enabled	Admin → Settings → Application Settings → MedInquiry → Enable FAQ Auto-Detection
Works for some products not others	FAQ records wrong status or wrong country/language scope	Check FAQ records — verify status = Active and scope matches Case Requests
Similar questions get no suggestion	Similarity threshold too high (e.g. 90%)	Contact Veeva Services to lower threshold to 65-70%
Wrong FAQ suggested	FAQ question__v text too generic	Rewrite FAQ question__v to be more specific — include product name and condition
Works in English not German	German-language FAQ records not created	Create German FAQ + Standard Response records with language__v = German

■ ■ **Best practice for FAQ question text:** Write it how an HCP would actually say it. Instead of "Drug X dosing guidelines in renal impairment" write "What is the correct dose of Drug X for a patient with kidney problems or reduced renal function?" — natural language matches incoming questions far better.

7. Adverse Event Handling

What Counts as an Adverse Event?

Scenario	AE?	Reason
Patient developed a rash after starting Drug X	YES	Adverse experience — capture regardless of suspected cause
Drug X did not work for my patient	YES	Lack of efficacy is reportable in many regulatory frameworks
Patient took double dose by mistake	YES	Overdose is always an AE regardless of whether harm occurred
Patient used Drug X during pregnancy	YES	Exposure in pregnancy — must capture and follow up
What is the dose of Drug X in elderly patients?	NO	Medical inquiry only — no patient harm mentioned
Drug X tablets look different this month	NO (PQC)	Product Quality Complaint — not an AE

Serious vs Non-Serious AE

The **serious__v** field determines regulatory reporting timelines. An AE is Serious if it results in: Death · Life-threatening condition · Hospitalisation (initial or prolonged) · Persistent disability or incapacity · Congenital anomaly or birth defect · Other medically important condition.

■ **Reporting timelines:** Serious unexpected AEs must be reported to regulatory authorities within **15 calendar days** of the **awareness_date__v**. Non-serious AEs are reported in periodic safety reports. The **awareness_date__v** field starts this clock — it must be set correctly on the day the company first hears about the AE.

Medical-Safety Connection — AE Transfer End to End

- 1 Specialist captures AE**
All fields completed: patient demographics, drug details (event_product__v), reaction details (event_reaction__v), seriousness, awareness date.
- 2 Specialist clicks "Transfer to Safety"**
User action on Adverse Event record — available once all required fields complete.
- 3 Medical-Safety Vault Connection fires**
Native Vault Connection packages all AE data and sends to Safety Vault via configured Integration Point.

4

Safety Vault receives Inbox Item

All AE data pre-populated in Inbox Item. PV specialist processes it: assigns safety case number, assesses seriousness.

5

Reconciliation flows back

Safety team sends reconciliation data back. `safety_case_number__v` populated in MedInquiry. Both teams see complete picture — no manual meetings needed.

8. Configuration Steps — Admin Navigation Reference

Feature Enable Paths

Feature	Navigation Path	Notes
Enable MedInquiry	Admin → Settings → Application Settings → MedInquiry Settings → Enable MedInquiry	Must be first — activates all MI objects
Medical Inquiry UI	Admin → Settings → Application Settings → MedInquiry → Enable Medical Inquiry UI	Activates single-form Case Intake
FAQ Auto-Detection	Admin → Settings → Application Settings → MedInquiry → Enable FAQ Auto-Detection	Threshold via Veeva Services only
Secure Response Link	Admin → Settings → Application Settings → MedInquiry → Enable Secure Document Link	Set expiry days on same page
Keyword Tracking	Business Admin → Objects → Medical Inquiry Keyword → Create keyword records	No toggle — just create records
Communication Platform (SCP)	Admin → Settings → Application Settings → MedComms Settings → Enable Communication Platform UI	MedComms feature — used for SRD sourcing

Configuration Phases — Correct Order

PHASE 1 — FOUNDATION

- **Enable MedInquiry in Application Settings**
 - Path: Admin → Settings → Application Settings
- **Configure Case Object Page Layout**
 - Path: Admin → Objects → case__v → Page Layouts
- **Configure Case Lifecycle (states, transitions, entry actions)**
 - Path: Admin → Configuration → Object Lifecycles
- **Configure Case SLA Rules (one record per priority)**
 - Path: Business Admin → Objects → Case SLA → Create
- **Configure User Permission Sets (Specialist, Supervisor, Viewer)**
 - Path: Admin → Users & Groups → Permission Sets

PHASE 2 — CASE INTAKE

- **Enable Medical Inquiry UI (single-form intake)**
 - Path: Admin → Settings → Application Settings → MedInquiry
- **Configure Case Intake object page layout**
 - Path: Admin → Objects → case_intake__v → Page Layouts
- **Configure Channel picklist values on Case**
 - Path: Admin → Configuration → Picklists → Channel picklist

PHASE 3 — FAQ LIBRARY (MOST CRITICAL)

- **Create FAQ records with question__v text per product/country/language**
 - Path: Business Admin → Objects → FAQ → Create
- **Create Standard Response records linked to each FAQ**
 - Path: Open FAQ → Related "Standard Responses" → Create
- **Link SRD documents to Standard Responses**
 - Path: Open Standard Response → Related "Standard Response Docs" → Create
- **Configure FAQ auto-detection (threshold via Veeva Services)**
 - Path: Admin → Settings → Application Settings

PHASE 4 — ADVERSE EVENTS

- **Configure Event object page layout (AE type with patient/product/reaction sections)**
 - Path: Admin → Objects → event__v → Page Layouts
- **Create Medical Inquiry Keyword records for safety terms**
 - Path: Business Admin → Objects → Medical Inquiry Keyword
- **Configure Medical-Safety Vault Connection (via Veeva Services)**
 - Path: Admin → Vault Connections

PHASE 5 — RESPONSE

- **Create Cover Letter template(s) with merge fields**
 - Path: Business Admin → Objects → Cover Letter Template
- **Configure Secure Response Link delivery**
 - Path: Admin → Settings → Application Settings → MedInquiry
- **Configure Case Response Approval Workflow (optional)**
 - Path: Admin → Configuration → Object Lifecycles → Case Response

PHASE 6 — INTEGRATIONS

- **Configure Medical-CRM Connection**
 - Path: Admin → Vault Connections → Medical-CRM
- **Configure Genesys Cloud telephony integration**
 - Path: Admin → Settings + Genesys Embeddable Framework config

■■■ **Important:** Veeva strongly recommends that Phases 3–6 be configured with Veeva Services support due to complex interdependencies. Some settings (FAQ similarity threshold, Medical-Safety Connection) cannot be configured without Veeva backend access.

9. Implementation Planning

16-Week Implementation Timeline

Week	Phase	Key Activities
1–3	Discovery & Design	Map current inquiry process, list all channels, define SLA requirements, identify top 50 FAQs, confirm AE reporting obligations
4–6	Foundation Build	Configure Case lifecycle, SLA rules, Permission Sets, Page Layouts, Case Intake UI
7–9	FAQ Library Build	Create FAQ records, Standard Responses, link SRD documents, configure auto-detection
10–11	AE & Integrations	Configure AE capture, Keyword Tracking, Medical-Safety Connection, CRM integration
12–13	Response Build	Configure Cover Letter templates, Secure Link delivery, Response Approval workflow
14–15	Testing & UAT	End-to-end testing all workflows, SLA calculations, AE transfer, FAQ matching. UAT with MI team.
16	Go-Live & Hypercare	Train specialists, go-live, 2-week hypercare support, 30-day monitoring

Pre-Implementation Preparation Checklist

Item to Prepare	Owner	Why Critical
List of all products MedInquiry will cover	Medical Affairs	Drives Product setup and FAQ scope
All countries and languages in scope	Medical Affairs	Drives FAQ language variants and SLA rules
Top 50 FAQs per product per country identified	MI Specialists	Without FAQs auto-detection never works
All SRD documents written, reviewed, approved in MedComms	Medical Writers	Cannot link SRDs until they exist in MedComms
SLA requirements defined per priority per country	Medical Affairs / Legal	Some markets have regulatory SLA obligations
AE reporting timelines confirmed with PV team	Pharmacovigilance	Drives urgency classification and awareness_date__v config
All MI specialists identified – user accounts created	IT / Admin	Needed before Permission Set and URS config
Response delivery method decided (secure link vs attachment)	IT / Medical Affairs	Affects cover letter and response configuration

Item to Prepare	Owner	Why Critical
Telephony channel confirmed (Genesys Cloud or manual)	IT	Genesys integration is a separate IT project track
Safety Vault ready – Medical-Safety Connection scoped	IT / Safety / Veeva Services	Connection requires both Vaults in same Veeva Domain

Common Mistakes to Avoid

Mistake	Impact	Prevention
Go-live with fewer than 20 FAQs	Auto-detection rarely fires — all inquiries are manual research	Build 50+ FAQs covering top questions before go-live
AE transfer not tested before go-live	First real AE transfer fails on a live case — compliance risk	Test Medical-Safety Connection with 10+ test AEs in UAT
SLA time unit configured incorrectly	Deadlines calculated in hours instead of business days	Test SLA calculation including weekends and business hours
Specialists not trained on AE capture before go-live	AEs missed or captured incorrectly in first weeks	Mandatory AE training + quick reference card on desks
Cover letter merge fields untested	Blank fields in response letters sent to HCPs	Test every merge field with actual Case data before go-live

■ **Go-live benchmark targets (measure at 60 days):** FAQ auto-detection rate >60% · SLA compliance rate >95% · 100% of AEs transferred to Safety on same day as capture · Zero inquiries handled outside the system.

10. System Architecture

How MedInquiry Connects to Other Systems

Connected System	Direction	What Flows	Connection Type
Vault Safety	MedInquiry → Safety (+ reconciliation back)	AE data as Inbox Items. Safety outcome reconciles back to MedInquiry.	Medical-Safety Vault Connection
Vault MedComms	MedComms → MedInquiry	Approved SRD documents accessible as Fulfillment Documents. Single source of truth.	PromoMats-Medical Vault Connection
Veeva CRM	CRM → MedInquiry	MSL/rep inquiry submissions auto-create Cases. Fulfillment status visible in CRM.	Medical-CRM Vault Connection

Connected System	Direction	What Flows	Connection Type
Vault Quality (QMS)	MedInquiry → Quality	Product Quality Complaints transferred for investigation.	Medical-Quality Vault Connection
Genesys Cloud	Telephony → MedInquiry	Incoming call triggers Case Intake form. Caller phone may pre-populate.	Genesys Embeddable Framework
External Web Form	Web → MedInquiry	Form submissions create Cases via REST API.	Vault REST API (POST /objects/case__v)

■■ **Architecture principle:** MedInquiry is the single hub for all medical inquiry data. Every channel feeds in. All other systems (Safety, Quality, MedComms, CRM) connect to it. This single-hub model ensures 100% inquiry capture, no siloed data, and a complete audit trail in one place. The value of each connection is multiplicative — each one eliminates at least one manual data transfer that previously required human intervention.